

After your extraction

RECOVERY GUIDE · ROUTINE, SURGICAL, & BONE GRAFT

Keep this guide handy
Call our office with any questions

Your tooth has been removed. Sometimes we also place a bone graft to preserve the shape of your jaw for a future implant, bridge, or denture. Most of the healing will happen on its own — your main job in the first few days is to protect the blood clot and give the tissue time to close.

TODAY

Rest. Gauze. Ice. Soft, cool foods. Don't disturb the site.

DAYS 1-3

Swelling peaks. Start saltwater rinses tomorrow. Pain tapers.

DAYS 4-7

Most discomfort gone. Diet expands. Watch for dry socket.

WEEK 2+

Tissue closes over. If you had a graft: full bone healing takes 3-4 months.

1 The first 24 hours (most important)

PROTECT THE CLOT

A soft, Jell-O-like blood clot forms in the socket. It's your body's natural bandage — losing it early is the main cause of "dry socket."

- ✗ No rinsing, swishing, or spitting for 24 hours — let saliva gently drain
- ✗ No straws, no vaping, no sucking motions for 72 hours
- ✗ No smoking for at least 72 hours — smoking roughly triples the risk of dry socket
- ✗ Don't poke the site with your tongue, finger, or toothpick

CONTROL BLEEDING

Bite firmly on the gauze we gave you for 30-45 minutes. Some oozing and pink-tinged saliva for the first day is normal.

 **If bleeding restarts:** fold a fresh gauze pad (or a damp black tea bag — the tannins help clotting), place it directly on the socket, and bite firmly without checking for 30 minutes. Keep your head elevated.

ICE FOR SWELLING

- 20 minutes on, 20 minutes off, for the first 24 hours
- Peak swelling is at 48-72 hours — not the day of surgery. This is normal.
- After 48 hours, switch to warm moist compresses to help swelling go down

EAT SOFT, COOL FOODS

GOOD CHOICES

- ✓ Yogurt, cottage cheese, pudding
- ✓ Smoothies (spoon, no straw)
- ✓ Mashed potatoes, applesauce
- ✓ Scrambled eggs, soft pasta
- ✓ Lukewarm soup, broth

AVOID FOR THE FIRST FEW DAYS

- ✗ Hot temperatures (can restart bleeding)
- ✗ Hard, crunchy, chewy, or sharp foods
- ✗ Chips, nuts, popcorn, crusty bread
- ✗ Carbonated and alcoholic drinks
- ✗ Chewing directly on the surgical side

REST

Take the rest of the day off. No work, school, or exercise. Avoid heavy lifting or strenuous activity for 48-72 hours — increased blood pressure can restart bleeding. Sleep with your head slightly elevated (an extra pillow) the first night.

2 Pain & swelling

Discomfort usually peaks at 48–72 hours, then tapers quickly. A combination of **ibuprofen + acetaminophen** is as effective as prescription opioids for most dental pain — and safer. See your **Medication Guide** for exact dosing.

- Start your pain medication *before* the numbness wears off
- Take ibuprofen with food to protect your stomach
- Stay ahead of pain — use scheduled doses for the first 2 days rather than waiting
- If an opioid was prescribed, only take it if ibuprofen + acetaminophen isn't enough

Days 2 through 7 and beyond

EXTRACTION RECOVERY · CONTINUED

3 Days 2 through 7

START SALTWATER RINSES TOMORROW

Mix ½ teaspoon of salt in 8 oz of warm water. Rinse gently 3–4 times a day, especially after meals. **Let the water fall out of your mouth** rather than forcefully spitting. Continue for 1–2 weeks.

i If you were prescribed a **chlorhexidine rinse** (Peridex, Paradex, Periogard), use it twice daily as directed, and use saltwater the rest of the day. Avoid eating or drinking for 30 minutes after rinsing.

ORAL HYGIENE

- Brush the rest of your teeth normally starting the day after surgery
- Avoid the extraction site directly for the first 3 days
- After day 3, a soft toothbrush can gently clean near (but not in) the site
- Keep flossing everywhere except right next to the surgical area

WHAT'S NORMAL VS. WHAT'S NOT

NORMAL (EXPECTED)

- ✓ Swelling peaking at 48–72h, then improving
- ✓ Mild bruising of cheek or jaw
- ✓ Yellow or whitish tissue in the socket (healing, not pus)
- ✓ Slight bad taste or smell for a few days
- ✓ Stiff jaw that slowly improves
- ✓ Sensitivity in neighboring teeth
- ✓ Slightly altered or "less sharp" sensation in lip, chin, or tongue that's gradually improving

CALL OUR OFFICE

- ✗ Severe pain on day 3–5 that medication doesn't touch at all (possible dry socket)
- ✗ Pain, swelling, or bad taste that returns *after* you'd started to feel better (possible infection)
- ✗ Bleeding that won't stop after 30–45 min of firm pressure
- ✗ Fever over 101°F
- ✗ Complete numbness (like dental freezing) in lip, chin, or tongue still present after 2 days

💡 Is it dry socket? Here's a simple test. If your pain medication brings pain down to a manageable level (around **2 or 3 out of 10**), it's *not* dry socket — you're on a normal healing curve. If the medication does **nothing at all** — the pain stays severe no matter what you take — that's the hallmark of dry socket. Call us; it's easily treated with a medicated dressing that brings fast relief.



Is it an infection? Watch the *direction* of your pain, not just how much it hurts. Days 2–4 are usually the peak. After that, each day should be a little better than the last. If you were feeling better and then the pain, swelling, or a bad taste suddenly comes back, that's the classic pattern for an infection. Call us.

i Why do these happen? Dry socket and infection are the most common complications we see — about **5–12% of cases combined** — and both are usually straightforward to treat. The mouth isn't sterile the way skin is. Unlike a cut on your arm, we can't scrub the site clean of bacteria before surgery — they're everywhere in the mouth. Even with sterile instruments and careful technique, some patients still develop one. The risk is higher if the tooth was already infected before we removed it.

i About nerve sensation: It's common for the lip, chin, or tongue to feel a little "off" or less sharp for a day or two after some extractions — this is usually from the nerve being bruised, and it almost always fully recovers on its own. What we want to know about is **complete numbness** (the feeling of still being frozen from dental anesthetic) that lasts more than 2 days.

SUTURES

If dissolvable sutures were placed, they'll fall out on their own over 7–14 days. If non-dissolvable sutures were placed, you'll come back for removal — typically 7–10 days after your procedure.

ONLY IF YOU HAD A BONE GRAFT

Your bone graft

A graft was placed to preserve the bone shape for a future implant, bridge, or denture fit. You'll also receive the **Bone Graft Recovery Guide**, and where it is stricter, it wins: **no tobacco for 1 month** and **no alcohol for 7 days** (not 72 hours), sutures dissolve over **7–10 days**, and Peridex is a **1-minute soak** for 1 week after surgery. A few things to expect:

- You may see small whitish-gray particles — this is graft material. **Do not pick at them.**
- A few grain-like bits in your saliva over the first few days is normal. Some loss is expected — you don't need to save them.
- **Don't probe with your tongue.** Pressure can shift the graft before it integrates.
- If you see a soft membrane at the site, or part of it comes loose, it's *not* an emergency. Call us and we'll check it at your follow-up.
- Full bone healing takes **3–4 months** before the site is ready for an implant or other restoration.
- If you wear a denture or partial that covers this area, **don't wear it until we adjust it** — pressure can disturb the graft.

ONLY IF THE BONE WAS SMOOTHED (ALVEOPLASTY)

Bone smoothing (alveoplasty)

Alongside your extraction, we reshaped the bone underneath — smoothing sharp edges or high points — so the area will be comfortable and ready for a future denture, partial, or implant. This adds a little to your recovery:

- Expect **slightly more swelling and soreness** than a routine extraction for the first 2–3 days.
- You may feel a small, sharp bony edge under the gum as it heals — usually this smooths out on its own as tissue and bone remodel over several weeks.
- If a small sharp piece (a "bone spicule") works its way through the gum weeks later, don't worry — it's a normal part of healing. Call us and we'll remove it quickly if it bothers you.
- Full bone remodeling takes 2–3 months.

5 Other things that may help

A few evidence-backed additions that support recovery:

- **Bromelain** (500 mg, 3× daily, 5–7 days) — a pineapple-derived enzyme with research support for reducing swelling and speeding recovery after oral surgery. *Skip if you take a blood thinner, have a bleeding disorder, or are allergic to pineapple.*
- **Sleep** — 7+ hours a night the first week. Sleep loss measurably slows healing.
- **Protein** — aim for 80–100g daily. Soft options: Greek yogurt, cottage cheese, eggs, protein smoothies (spoon only), soft fish.
- **Hydration** — plenty of water. Not sugary drinks, not alcohol.
- **No nicotine** — smoking, vaping, and nicotine pouches all slow healing and raise the risk of dry socket. The longer you can stay off, the better.
- **No alcohol for at least 72 hours** — interacts with medications and slows healing.

i Supplements not on this list (arnica, megadose vitamin C, "immune boosters," CBD) don't have strong evidence for healing after oral surgery. Save your money.

When to call us

- Severe pain on day 3–5 that your medication doesn't help at all (possible dry socket)
- Pain, swelling, or bad taste that returns after you'd started to feel better (possible infection)
- Heavy bleeding that won't stop after 30–45 min of firm gauze pressure
- Swelling spreading to your neck or around your eye
- Fever over 101°F
- Complete numbness in lip, chin, or tongue (like dental freezing) still present after 2 days
- Jaw stiffness getting worse (not better) after day 5

URGENT — DON'T WAIT

- Difficulty breathing or swallowing
- Rapidly spreading swelling to your neck, under your tongue, or around your eye

Call our office right away for the on-call provider. If you truly cannot breathe or swallow, call 911.

Most calls are easy to resolve. If you're unsure whether something is normal, call us — we'd rather answer than have you worry about it.

Acknowledgment & signature

I acknowledge that I have received and understand these instructions, and all my questions have been answered. I agree to follow them and to call the office with any questions or concerns.

PATIENT SIGNATURE

DATE

PATIENT PRINTED NAME

DATE OF BIRTH

PARENT / LEGAL GUARDIAN / CAREGIVER SIGNATURE (IF SIGNING FOR THE PATIENT)

DATE

PRINTED NAME

RELATIONSHIP TO PATIENT